PEER REVIEW HISTORY

BMJ Open publishes all reviews undertaken for accepted manuscripts. Reviewers are asked to complete a checklist review form (**http://bmjopen.bmj.com/site/about/resources/checklist.pdf**) and are provided with free text boxes to elaborate on their assessment. These free text comments are reproduced below.

ARTICLE DETAILS

TITLE (PROVISIONAL)	Returning to clinical work and doctors' personal, social and organisational needs: a systematic review
AUTHORS	Attoe, Chris; Matei, Raluca; Thompson, Laura; Teoh, Kevin; Cross, Sean; Cox, Tom

VERSION 1 – REVIEW

REVIEWER	Hoffman, Rebekah University of Wollongong, Graduate School of Medicine
REVIEW RETURNED	25-Jul-2021

GENERAL COMMENTS	This is a quality study examining the important topic around return to work at a systems and individual level. I hope it leads to further research in the area.

REVIEWER	Ashokka, Balakrishnan National University Singapore Yong Loo Lin School of Medicine, Anaesthesia
REVIEW RETURNED	23-Aug-2021

GENERAL COMMENTS	Returning to clinical work and doctors' personal, social and organisational needs: a systematic review This is an important topic and authors have made an effort to collate existing body of evidence to understand factors influencing the return of doctors to work The data quotes chiefly general practice and more on women. The authors have attempted to synthesise quantitative and qualitative studies and have provided broad conceptualisation of the inherent issues as reported by the doctors themselves who have been absent and had returned. Reasons for absence can range from: - Illness that they return from - Social roles like maternity that they continue to support while they return - Life crises like substance use, nervous breakdown that they will continue or struggle to cope with once they return to work also Hence clarifying or quantifying what absence is and how long is considered a label of "absence" or evaluating what considered a criterion to full return to work might be heterogeneous. The authors could bring this in introduction/ discussion or state as limitations in data gathering etc The authors have done good work in trying to synthesise data from both quantitative and qualitative and we understand the source papers are self-reported primary papers.

	While the authors chose to exclude papers with third person reporting of outcomes and have kept to data from self reports, we are not sure if the supervisors' inputs are more objective in stating if the return of work effort was indeed a personal success or organisational nightmare. They could keep that option open for suggestions for future research The quality indices especially for the qualitative are good and as authors have noted, its more robust on the qualitative slant to the paper. As it is more of a narrative or paradigms of the various domains or components that are to be addressed, may be a figure conceptualising this framework might add more readability. Infographics seem to be the in vogue and would add value if the authors want to consider that. Implication's paragraph is well written and is a good message Table 5 : good level of categorisation into a 3*3 table Table 6 igloo framework Is nice to summarise Scope for improvements: Methodology: Page6/30- line 23-24: RM had screened only 20%. Its customary to have either two individual investigators individually doing search and then summarising the search output and a third judge/ moderating coder decides the final say on conflicts or doubtful on what to include on full text. Alternatively, it is done with half the chosen full text by one coder and the other half by other coder and then conflicts are resolved in team meeting. Sometimes librarians provide technical search and coders provide content screening. The authors can clarify what was actually done in reply to reviewers and if these are indeed limitations to the research dynamics with pandemic restrictions, they can then include in the review limitations Page 6/30 – line 38 : Non-English language was excluded. This can be stated as limitations and such each continent might differ in the resource poor settings. All papers quoted are from first-world contexts Page 18-line 20: general practices- what were the other proportions- the issues might be very different from surgical practice or operation theatre related fields where relief or time off is very limited – can included in weaknesses Discussion: (authors can work on this segment to match it the good work they have done till results section) Page 20/30: line 17 "drawing on wider literature may help" The authors need to commit to actually appraising the review data with what evidence is out there and inform the reader that the present work is state of art and it hence: - Clarifies the research question - Addresses the dearth in the existing body of evidence - The reasons for the difference in what they found disproving present beliefs - Or checking in management and HR journals on this where a lot is described, if that helps

	When the data sources are so heterogeneous, yes the pooling of effect size or what works well is not possible. Hence the review would form a narrative of the present topic. How this informs the next team of researchers is how the authors suggest what next such as: - Need to dive further deeper or go broader - Is there a benefit from other procedural and acuta care areas and how that dynamics would be? - What would they have done in retrospect if they had to redo this whole process as the team of authors would be best suited in this topic after immersing in this for 2 years or so Language, text and grammar: Page 15/30 line 46: negatives views Page 17/30- line 16: suggested by three studies each Page 20/30 Line 44-46: the line needs rephrasing “Doctors taking absence and planning to return may benefit from being able to prospectively and proactively consider their needs according to current evidence and this framework, as well as biopsychosocial understandings of their health needs” Line 47: This may empower: what may?: prospectively considering or having evidence framework In addition can consider the following: 1. Page 5, Line 23-25: more details should be provided for the inter-rater reliability of reviewing the abstracts and the articles. 2. Page 7, Line 1-3: Good to indicate which co-authors had experience with RTW, and whether all authors are familiar with this topic. 3. Table 3 - MERQI score: details should be provided for the scoring system. 4. Under Results (P13-16), it will be easier to read if the results are presented in tables, including which studies are linked to which key findings. 5. Under Principle Finding, the 1st paragraph seems to be repeating the info from the previous 3 pages. 6. Under "Strengths and weakness", for the qualitative studies in the 2nd paragraph, these studies did have flaws, as the authors described in Table 4. Please add a couple of sentences to include the limitations of these qualitative studies.

VERSION 1 – AUTHOR RESPONSE

Reviewer Reports:

Reviewer: 1
Comments to the Author:

This is a quality study examining the important topic around return to work at a systems and individual level. I hope it leads to further research in the area.

We thank reviewer 1 for their kind comments.

Reviewer: 2
Comments to the Author:

Returning to clinical work and doctors' personal, social and organisational needs: a systematic review. This is an important topic and authors have made an effort to collate existing body of evidence to understand factors influencing the return of doctors to work The data quotes chiefly general practice and more on women.
The authors have attempted to synthesise quantitative and qualitative studies and have provided broad conceptualisation of the inherent issues as reported by the doctors themselves who have been absent and had returned.
Reasons for absence can range from:
- Illness that they return from
- Social roles like maternity that they continue to support while they return
- Life crises like substance use, nervous breakdown that they will continue or struggle to cope with once they return to work also
Hence clarifying or quantifying what absence is and how long is considered a label of "absence" or evaluating what considered a criterion to full return to work might be heterogeneous. The authors could bring this in introduction/ discussion or state as limitations in data gathering etc The authors have done good work in trying to synthesise data from both quantitative and qualitative and we understand the source papers are self-reported primary papers.
While the authors chose to exclude papers with third person reporting of outcomes and have kept to data from self reports, we are not sure if the supervisors' inputs are more objective in stating if the return of work effort was indeed a personal success or organisational nightmare. They could keep that option open for suggestions for future research. The quality indices especially for the qualitative are good and as authors have noted, its more robust on the qualitative slant to the paper.
As it is more of a narrative or paradigms of the various domains or components that are to be addressed, may be a figure conceptualising this framework might add more readability. Infographics seem to be the in vogue and would add value if the authors want to consider that.
Implication's paragraph is well written and is a good message Table 5 : good level of categorisation into a 3*3 table Table 6 igloo framework Is nice to summarise

We thank the reviewer for their kind and considered comments. In response we have added considerations to the "Future research" and "Strengths and weaknesses" sections (pages 15-17) in relation to considering length of absence and the heterogeneity of doctors as a group.

We believe that Tables 5 and 6 are the most succinct and digestible ways to present the findings for readers. However, should the editorial office be keen for us to include a figure/infographic we would happily reconsider.

Scope for improvements:

Methodology:
Page6/30- line 23-24: RM had screened only 20%. Its customary to have either two individual investigators individually doing search and then summarising the search output and a third judge/ moderating coder decides the final say on conflicts or doubtful on what to include on full text. Alternatively, it is done with half the chosen full text by one coder and the other half by other coder and then conflicts are resolved in team meeting. Sometimes librarians provide technical search and coders provide content screening. The authors can clarify what was actually done in reply to

reviewers and if these are indeed limitations to the research dynamics with pandemic restrictions, they can then include in the review limitations

Thank you for this comment and clarification. We have added additional detail to the Methods on page 4 relating to screening. Indeed only 20% were reviewed by RM and 20% at the full text stage by SC due to resource limitations. Consequently, we have added this into the strengths and weaknesses section of the Discussion alongside more methodological limitations (page 15-16).

Page 6/30 – line 38 : Non-English language was excluded. This can be stated as limitations and such each continent might differ in the resource poor settings. All papers quoted are from first-world contexts

We have added this to the limitations on page 15-16, as well as clarifying where the included studies originated from in the abstract to aid interpretation of findings.

Page 18-line 20: general practices- what were the other proportions- the issues might be very different from surgical practice or operation theatre related fields where relief or time off is very limited – can included in weaknesses

We have added this to the limitations on page 15-16 regarding how specialty and setting (among other factors) may affect the needs of returning doctors.

Discussion: (authors can work on this segment to match it the good work they have done till results section) Page 20/30: line 17 “drawing on wider literature may help”
The authors need to commit to actually appraising the review data with what evidence is out there and inform the reader that the present work is state of art and it hence:
- Clarifies the research question
- Addresses the dearth in the existing body of evidence
- The reasons for the difference in what they found disproving present beliefs
- Or checking in management and HR journals on this where a lot is described, if that helps

Thank you for these comments, while we were trying to maintain brevity and adhere to BMJ guidelines we welcome the opportunity to expand on these points. As such we have changed the start of the Discussion to clarify the research question, we have added to the ‘Relation to other literature’ section in terms of the dearth of evidence, how the findings relate to the literature, and further discussion on the wider literature that we drew on (the IGLOO model). We hope this makes this section feel more discursive.

When the data sources are so heterogeneous, yes the pooling of effect size or what works well is not possible. Hence the review would form a narrative of the present topic.
How this informs the next team of researchers is how the authors suggest what next such as:
- Need to dive further deeper or go broader
- Is there a benefit from other procedural and acuta care areas and how that dynamics would be?
- What would they have done in retrospect if they had to redo this whole process as the team of authors would be best suited in this topic after immersing in this for 2 years or so

Thank you again for these comments. We have added another paragraph to the ‘Future research’ section which broadens our ideas presented as suggested.

Language, text and grammar:
Page 15/30 line 46: negatives views
Page 17/30- line 16: suggested by three studies each

Page 20/30
Line 44-46: the line needs rephrasing
“Doctors taking absence and planning to return may benefit from being able to prospectively and proactively consider their needs according to current evidence and this framework, as well as biopsychosocial understandings of their health needs”
Line 47: This may empower: what may?: prospectively considering or having evidence framework

Thank you again for these comments. We have corrected language, grammar and rewritten sentences as suggested.

In addition can consider the following:
1. Page 5, Line 23-25: more details should be provided for the inter-rater reliability of reviewing the abstracts and the articles. More information added.
2. Page 7, Line 1-3: Good to indicate which co-authors had experience with RTW, and whether all authors are familiar with this topic. Role of authors clarified in Methods.
3. Table 3 - MERQI score: details should be provided for the scoring system. Details added in text before Table 3 and to the table itself.
4. Under Results (P13-16), it will be easier to read if the results are presented in tables, including which studies are linked to which key findings. We believe that we may not be able to add more tables based on the BMJ Open guidelines(?) and that this may duplicate information in Table 2, Table 5, and in text and cause confusion by presenting the same information in another way. Consequently we have decided to not add another table but should the editors wish us to do this we will happily oblige.
5. Under Principle Finding, the 1st paragraph seems to be repeating the info from the previous 3 pages. Indeed our interpretation of the BMJ guidelines was to begin the Discussion section with a short summary of the principal findings.
6. Under "Strengths and weakness", for the qualitative studies in the 2nd paragraph, these studies did have flaws, as the authors described in Table 4. Please add a couple of sentences to include the limitations of these qualitative studies. We have added this into the strengths and weaknesses section for clarity (top of page 15).

VERSION 2 – REVIEW

REVIEWER	Ashokka, Balakrishnan National University Singapore Yong Loo Lin School of Medicine, Anaesthesia
REVIEW RETURNED	10-Feb-2022

GENERAL COMMENTS	The Authors have done a good revamp and reads well Most of the queries are addressed and explanations stated for what isnt Appreciate if they could address these minor prose corrections Best wishes ========== Page 6 of 62 Line 19: between-group Page 12-62 Line 4 & Line 17 : can remove “ further” Line 14-16: Most commonly in six studies this was self efficacy .. : The two lines can be rephrased as its not clear what “this” means Line 42- Returner’s needs… were Page 13 of 62:

	Line 55-56 : These needs were (as the previous line describes in plural "Highlighted needs") Page 14 0f 62 Line 49-50: please remove most commonly Page 16 of 62 Line 15-16…experiencing RTW.. (I am thinking that RETRUN TO WORK is a process or phenomenon not something to be quoted as experiencing) Overall can search/ find “THIS” in the manuscript and see if we can minimise the usage.. The discussion starts with a This too 17 of 62 Line 22: "needs are improving with added the depth"